

CASE STUDY

They asked for paperless. The patients were still **Waiting.**

They asked us to digitise patient intake and get rid of the paper. The scope: paperwork was never the bottleneck — patients waited because nobody could see bed and clinician capacity in real time, so beds sat empty while the queue grew.

Real Problem

“Digitise our patient intake and admissions forms — get rid of the paper.”

Scope

Paper wasn’t slowing patients down; invisible capacity was. Beds cleared without anyone knowing, and admitted patients waited in the ED for rooms that were already empty.

01. The Situation

A private hospital group ran admissions on paper and coordinated capacity through a morning huddle and a spreadsheet. Admitted patients boarded in the emergency department while, two floors up, beds sat clean and unclaimed. Nobody was doing anything wrong — the information about who was leaving, which bed was free, and which clinician was available simply arrived hours after it was useful.

02. What We Found

Our diagnostic surfaced three compounding issues.

- 1. **Discounting with no guardrails.** Reps and store managers discounted at will; off-policy discounts were invisible until month-end.
- 2. **Promotions that stacked.** Online codes combined with markdowns; nobody governed promo-on-promo margin.
- 3. **Trade-desk deals in spreadsheets.** Terms drifted and accounts were under-billed against agreed pricing.

1 in 4

By 2024, more than a quarter of admitted patients waited four hours or more for an inpatient bed — even outside peak months. Hospitals that gained live visibility and predictive discharge have cut ED boarding time by 46% and discharge processing time by 32%, without adding a single bed. The constraint is rarely the number of beds. It’s knowing which ones are free.

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03. What We Did

We did build the paperless form — it was the ask, and it produced clean data at source. Then we used that data for what it was actually worth.

Capture at the door

Intake and admissions rebuilt as a PowerApps form —the paperless ask, delivered on day one, and the source of clean, instant data.

Make capacity visible

A live flow board shows every bed, its status and expected discharges—pulled from the patient-admin system, updated as it happens rather than at handover.

Automate assignment

PowerAutomate matches patients to beds and clinicians, triggers cleaning and discharge workflows, and escalates the moment flow breaks.

A NOTE ON SCOPE

We did not replace the patient-admin system or the electronic health record. PowerPlatform sat on top as the coordination and visibility layer, integrated with the system of record —any engagement claiming otherwise is one to walk away from.

04. How We Rolled It Out

We piloted on one ward, proved the reduction in boarding time, then scaled — with the charge nurses and bed managers in the build, because the workflow had to match how the floor actually runs. The paperless form shipped early, so the original ask was met while the real problem was being solved.

05. What An Engagement Like This Delivers

We started with a margin diagnostic across all three channels to quantify where money was escaping. Only then did we build.

~40%

Less time waiting for a bed

Admitted patients moved out of the ED faster.

~0.5d

Lower average length of stay

Discharges planned ahead, not discovered late.

~70%

Less admin time on admissions

No paper, no re-keying, no phone chase.

The win wasn’t a paperless form. It was patients moving, because someone could finally see the whole floor.

Bring us the problem

No proposal until we understand the problem.